Name	: Ms. NIKITA CHUDHARY		
Lab No.	: 474764803	Age	: 27 Years
Ref By	: DR NITIN NAHAR	Gender	: Female
Collected	: 9/7/2025 4:56:00PM	Reported	: 10/7/2025 6:31:50PM
A/c Status	: P	Report Status	: Final
Collected at	: BHOPAL CC-82 Mr Rachel V John Pata So Vitus John Mig 26 Graund,Indrapuri, Phone: 8770817968	Processed at	: LPL - Bhopal Lab II Plot No.05, Mandakini Housing Society, Nea Apurti Shopping Mall, Kolar Main Road, Bhopal, M.P. - 462042

Test Report

Test Name	Results	Units	Bio. Ref. Interval
HEMOGRAM (SLS Method, Sheath Flow DC Detection Method, Fluorescent Flow Cytometry, Calculated & Capillary Photometry)			
Hemoglobin	13.10	g/dL	12.00 - 15.00
Packed Cell Volume (PCV)	39.50	%	36.00 - 46.00
RBC Count	4.47	mill/mm3	3.80 - 4.80
MCV	88.40	fL	83.00 - 101.00
Mentzer Index	19.8		
MCH	29.30	pg	27.00 - 32.00
MCHC	33.20	g/dL	31.50 - 34.50
Red Cell Distribution Width (RDW)	11.90	%	11.60 - 14.00
Total Leukocyte Count (TLC)	7.33	thou/mm3	4.00 - 10.00
Differential Leucocyte Count (DLC)			
Segmented Neutrophils	49.10	%	40.00 - 80.00
Lymphocytes	**41.90**	%	20.00 - 40.00
Monocytes	6.00	%	2.00 - 10.00
Eosinophils	2.50	%	1.00 - 6.00
Basophils	0.50	%	<2.00
Absolute Leucocyte Count			
Neutrophils	3.60	thou/mm3	2.00 - 7.00
Lymphocytes	**3.07**	thou/mm3	1.00 - 3.00
Monocytes	0.44	thou/mm3	0.20 - 1.00
Eosinophils	0.18	thou/mm3	0.02 - 0.50

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Test Report

Test Name	Results	Units	Bio. Ref. Interval
Basophils	0.04	thou/mm3	0.02 - 0.10
Platelet Count	255	thou/mm3	150.00 - 410.00
Mean Platelet Volume	9.1	fL	6.5 - 12.0
E.S.R.	6	mm/hr	0.00 - 20.00

Comment

In anaemic conditions Mentzer index is used to differentiate Iron Deficiency Anaemia from Beta- Thalassemia trait. If Mentzer Index value is >13, there is probability of Iron Deficiency Anaemia. A value <13 indicates likelihood of Beta- Thalassemia trait and Hb HPLC is advised to rule out the Thalassemia trait.

Note

1. As per the recommendation of International council for Standardization in Hematology, the differential leucocyte counts are additionally being reported as absolute numbers of each cell in per unit volume of blood
2. Test conducted on EDTA whole blood

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Test Report

Test Name	Results	Units	Bio. Ref. Interval
LIVER PANEL,SCREEN, SERUM			
AST (SGOT)	16.5	U/L	<35
ALT (SGPT)	13.3	U/L	<35
Alkaline Phosphatase (ALP)	55.90	U/L	30 - 120
Bilirubin Total	1.06	mg/dL	0.30 - 1.20
Bilirubin Direct	0.18	mg/dL	<0.2
Bilirubin Indirect	0.88	mg/dL	<1.10
Total Protein	7.49	g/dL	6.40 - 8.30
Albumin	4.60	g/dL	3.50 - 5.20
Globulin(Calculated)	2.89	gm/dL	2.0 - 3.5
A : G Ratio	1.59		0.90 - 2.00
KIDNEY PANEL; KFT,SERUM			
Creatinine	0.73	mg/dL	0.51 - 0.95
GFR Estimated	116	mL/min/1.73m2	>59
GFR Category	G1		
Urea	18.66	mg/dL	17.00 - 43.00
Urea Nitrogen Blood	8.71	mg/dL	6.00 - 20.00
BUN/Creatinine Ratio	12		
Uric Acid	4.81	mg/dL	2.60 - 6.00
Total Protein	7.49	g/dL	6.40 - 8.30

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Test Report

Test Name	Results	Units	Bio. Ref. Interval
Albumin	4.60	g/dL	3.50 - 5.20
Globulin(Calculated)	2.89	gm/dL	2.0 - 3.5
A : G Ratio	1.59		0.90 - 2.00
Calcium, Total	9.92	mg/dL	8.60 - 10.30
Phosphorus	4.34	mg/dL	2.40 - 4.40
Sodium	137.31	mEq/L	136.00 - 146.00
Potassium	4.86	mEq/L	3.50 - 5.10
Chloride	103.66	mEq/L	101.00 - 109.00

Note

1. Estimated GFR (eGFR) calculated using the 2021 CKD-EPI creatinine equation and GFR Category reported as per KDIGO guideline 2012.
2. eGFR category G1 or G2 does not fulfil the criteria for CKD, in the absence of evidence of kidney damage
3. The BUN-to-creatinine ratio is used to differentiate prerenal and postrenal azotemia from renal azotemia. Because of considerable variability, it should be used only as a rough guide. Normally, the BUN/creatinine ratio is about 10:1

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Test Report

Test Name	Results	Units	Bio. Ref. Interval
ANTI CCP (CYCLIC CITRULLINATED PEPTIDE), SERUM (ECLIA) Result Rechecked, Please Correlate Clinically.	<8.0	U/mL	<17.00

Note

1. Sensitivity of this assay is 70.6% and specificity is 98.2%
2. Specificity of Anti CCP antibodies in Juvenile arthritis patients has not been established

Comments

Anti CCP antibodies are useful for evaluating patients suspected of Rheumatoid arthritis. Positive results occur in 60-80% of Rheumatoid arthritis patients depending on disease severity. The positive predictive value of Anti CCP antibodies for Rheumatoid arthritis is far greater than Rheumatoid factor. False positive results are uncommon. Upto 30% patients with seronegative Rheumatoid arthritis also show Anti CCP antibodies.

Clinical Uses

- For diagnosis of early Rheumatoid arthritis - Anti CCP antibodies are detected in approximately 50-60% patients of Rheumatoid arthritis usually after 3-6 months of symptoms
- Prediction of severity of disease - Early Rheumatoid arthritis patients with Anti CCP positivity may develop a more erosive form of the disease as compared with Anti CCP negative patients
- To differentiate elderly onset Rheumatoid arthritis from Polymyalgia rheumatica and erosive SLE

RHEUMATOID FACTOR (RA), SERUM (Immunoturbidimetry)	<10.00	IU/mL	<14.00
VITAMIN B12; CYANOCOBALAMIN, SERUM (ECLIA)	**142.00**	pg/mL	211.00 - 946.00

Notes

1. Interpretation of the result should be considered in relation to clinical circumstances.
2. It is recommended to consider supplementary testing with plasma Methylmalonic acid (MMA) or plasma homocysteine levels to determine biochemical cobalamin deficiency in presence of clinical suspicion of deficiency but indeterminate levels. Homocysteine levels are more sensitive but MMA is more specific
3. False increase in Vitamin B12 levels may be observed in patients with intrinsic factor blocking antibodies, MMA measurement should be considered in such patients

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Test Report

Test Name	Results	Units	Bio. Ref. Interval

4. The concentration of Vitamin B12 obtained with different assay methods cannot be used interchangeably due to differences in assay methods and reagent specificity

VITAMIN D, 25 - HYDROXY, SERUM (ECLIA)	**67.50**	nmol/L	75.00 - 250.00

Interpretation

LEVEL	REFERENCE RANGE IN nmol/L	COMMENTS
Deficient	< 50	High risk for developing bone disease
Insufficient	50-74	Vitamin D concentration which normalizes Parathyroid hormone concentration
Sufficient	75-250	Optimal concentration for maximal health benefit
Potential intoxication	>250	High risk for toxic effects

Note

- The assay measures both D2 (Ergocalciferol) and D3 (Cholecalciferol) metabolites of vitamin D.
- 25 (OH)D is influenced by sunlight, latitude, skin pigmentation, sunscreen use and hepatic function.
- Optimal calcium absorption requires vitamin D 25 (OH) levels exceeding 75 nmol/L.
- It shows seasonal variation, with values being 40-50% lower in winter than in summer.
- Levels vary with age and are increased in pregnancy.
- A new test Vitamin D, Ultrasensitive by LC-MS/MS is also available

Comments

Vitamin D promotes absorption of calcium and phosphorus and mineralization of bones and teeth. Deficiency in children causes Rickets and in adults leads to Osteomalacia. It can also lead to Hypocalcemia and Tetany. Vitamin D status is best determined by measurement of 25 hydroxy vitamin D, as it is the major circulating form and has longer half life (2-3 weeks) than 1,25 Dihydroxy vitamin D (5-8 hrs).

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Test Report

Test Name	Results	Units	Bio. Ref. Interval

Decreased Levels

- Inadequate exposure to sunlight
- Dietary deficiency
- Vitamin D malabsorption
- Severe Hepatocellular disease
- Drugs like Anticonvulsants
- Nephrotic syndrome

Increased levels

Vitamin D intoxication

Dr.Swapnil Gupta
MD, Pathology
Chief of Laboratory
Dr Lal PathLabs Ltd

Dr Kiran Bhargava Pathak
MD, Pathology
Chief of Laboratory
Dr Lal PathLabs Ltd

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MC-2113

Test Report

Test Name	Results	Units	Bio. Ref. Interval
HLA - B27 (Flow Cytometry)			
HLA-B27, Disease Association	Negative		

Interpretation

RESULT	REMARKS
Negative	No expression of HLA-B27
Indeterminate	Grey zone (within 10 channels of cutoff)
Positive	Expression of HLA-B27 detected

Note

1. Test conducted on EDTA / Heparin / ACD whole blood
2. Anti HLA-B27 antibody cross reacts with several other antigens of HLA-B group specially HLA- B7 leading to false positive results
3. In cases of HLA-B27 negative Spondyloarthropathy, further testing recommended is HLA-C*06 Positive in Psoriatic arthritis) and HLA-B*07 (positive in Juvenile and Undifferentiated Spondyloarthropathy).

Comments

HLA-B27 is a major histocompatibility complex (MHC) class 1 molecule which is considered to be an important criteria for classification of Axial spondyloarthropathies and helps in establishing an early diagnosis of Ankylosing spondyloarthropathies before the development of irreversible deformity. In 2009, the Assessment of Spondyoarthritis International Society (ASAS) proposed new criteria for classification of Axial spondyloarthritis which are applicable to individuals with back pain $\geq$ 3 months and age of onset < 45 years. One important criteria is the presence of HLA-B27.

DISEASE	RISK IN %
Ankylosing spondyloarthropathies	89
Reiters syndrome	79
Juvenile Rheumatoid arthritis	42
Normal population	8

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MC-2113

Test Report

Test Name	Results	Units	Bio. Ref. Interval

DMC – 117735

DMC – 46663

Dr Beena Chandrasekhar
PhD, Life Sciences
Technical Director - Flowcytometry
NRL - Dr Lal PathLabs Ltd

Dr. Nagarjun Sai Jaine
MD, Pathology
Fellowship in Hematopathology (AIIMS)
Consultant- Hemato-Oncopathology & Flowcytometry
NRL - Dr Lal PathLabs Ltd

Dr Sunanda
MD, Pathology
Sr. Consultant Pathologist - Hematology & Immunology
NRL - Dr Lal PathLabs Ltd

----------End of report ----------

IMPORTANT INSTRUCTIONS

•Test results released pertain to the specimen submitted.•All test results are dependent on the quality of the sample received by the Laboratory.•Laboratory investigations are only a tool to facilitate in arriving at a diagnosis and should be clinically correlated by the Referring Physician .•Report delivery may be delayed due to unforeseen circumstances. Inconvenience is regretted.•Certain tests may require further testing at additional cost for derivation of exact value. Kindly submit request within 72 hours post reporting.•Test results may show interlaboratory variations.•The Courts/Forum at Delhi shall have exclusive jurisdiction in all disputes/claims concerning the test(s) & or results of test(s).•Test results are not valid for medico legal purposes.•This is computer generated medical diagnostic report that has been validated by Authorized Medical Practitioner/Doctor.•The report does not need physical signature.

(#) Sample drawn from outside source.

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Tel: +91-11-49885050,Fax: - +91-11-2788-2134, E-mail: lalpathlabs@lalpathlabs.com

National Reference lab, Delhi, a CAP (7171001) Accredited, ISO 9001:2015 (FS60411) & ISO 27001:2013 (616691) Certified laboratory.

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